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Project #049: Forum-Beyond the Bedside: How Interdisciplinary Rounds Streamline ICU Care and Transitions

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Abstract

Daily patient rounds in the Cardiovascular ICU, involved the primary medical team, primary nurse, and pharmacy. However, there was no established process to include other key team members such as physical and occupational therapy (PT/OT), registered dietitians (RD), case management, or nursing leadership. As a result, quality metrics and transition planning were inconsistently addressed. The unit experienced elevated CLABSI rates and increased case mix index (CMI) adjusted length of stay (LOS).

“P5 Roundup” launched in August 2024 to include the full interdisciplinary team. Rounds aimed to improve quality outcomes, promote mobility, and streamline transitions. A whiteboard tool guided discussion and then PT/OT and case management were added via PDSA to reduce LOS and strengthen discharge planning. A standardized EHR tool was developed to support consistent information sharing. CLABSI rates dropped 60% to 0.6 and CMI-adjusted LOS decreased 21% to 1.1 days. Improvements in therapy metrics contributed to LOS reduction. Structured interdisciplinary rounds improve outcomes and provide a replicable model for enhancing care quality and transitions in critical care.

Introduction

Within the CVICU, three primary medical teams conducted daily patient rounds with pharmacy and the primary nurse. Twice-weekly “Progression Rounds” included a broader interdisciplinary team and focused on transitions out of the ICU. However, these lacked structured discussion of quality metrics and were often lengthy due to infrequent interdisciplinary collaboration.

Interdisciplinary rounds are widely recognized to improve communication, patient safety, and care coordination in critical care settings. Literature supports structured rounding tools to improve consistency, accountability, and team engagement. Previous work has shown to decrease delays in care transitions and length of stay when interdisciplinary participation is standardized.

The “P5 Roundup” model was developed to close gaps in quality, functional recovery outcomes, and care coordination by ensuring daily interdisciplinary collaboration.

Aims:

- Improve quality in patient care by reducing CLABSI rate
- Decrease CMI-adjusted LOS by improving functional recovery outcomes and strengthening transition planning

Methods

Prior to initiating P5 Roundup, unit medical and nursing leadership collaborated to determine the team members, rounding tools, and future direction. To support adoption of this significant practice change, buy-in from stakeholders was vital to success. Implementation followed a phased approach.

A whiteboard was utilized as a tool to guide discussion and display information such as, vascular access, urinary catheters, mechanical circulatory support devices, diet, and mobility.

CLABSI rates and length of stay were the primary outcome measures chosen to determine success. Functional recovery measures that contribute to length of stay were also identified.

At the start, the P5 Roundup team consisted of nursing leadership, the medical team, pharmacy, and RD. Due to increased frequency of rounds, it was important to streamline the process and shorten the amount of time spent in rounds as much as possible. As efficiency improved, PT/OT, case management, and infection control were added to address transitions out of the ICU, LOS, and review infection concerns. A rounding tool in the EHR was developed to streamline information presented during rounds.

Outcomes

Data was reviewed from the same time period of the previous year to account for acuity fluctuations. From August 2024 to December 2024, the average CLABSI rate decreased 60% to 0.6 [Figure 1]. There was a 21% decrease in average LOS, despite a 22% increase in CMI [Figure 2] and improvement in the following functional recovery outcome measures:

- Time from PT/OT consult order to first PT or OT visit [Figure 3]
- Mobility score [Figure 4]
 - Patients with improved AM-PAC 6 Clicks basic mobility score (first to last score)
- Patients discharged to inpatient rehabilitation facility [Figure 5]

Data

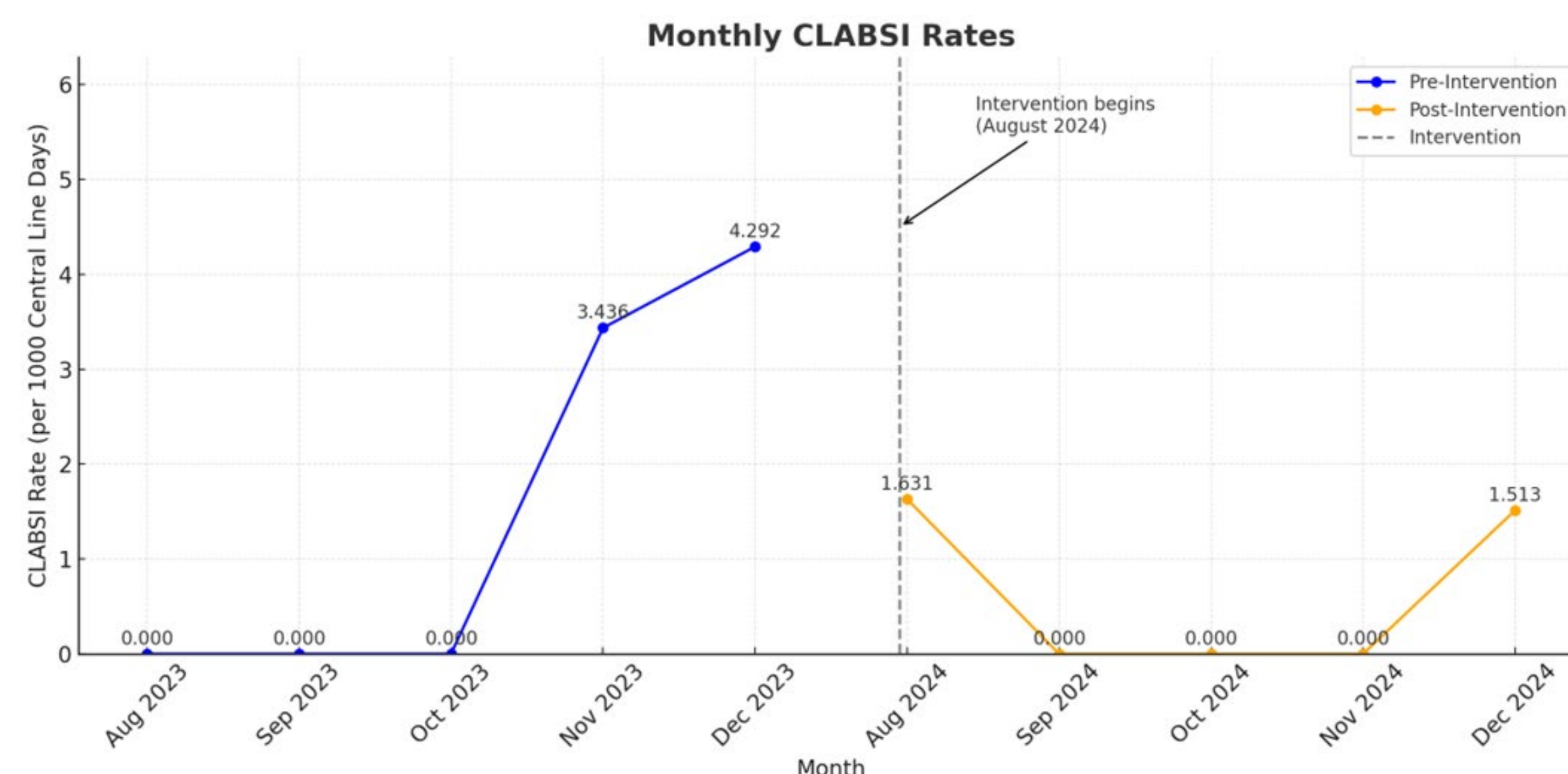


Figure 1. Monthly CLABSI rates

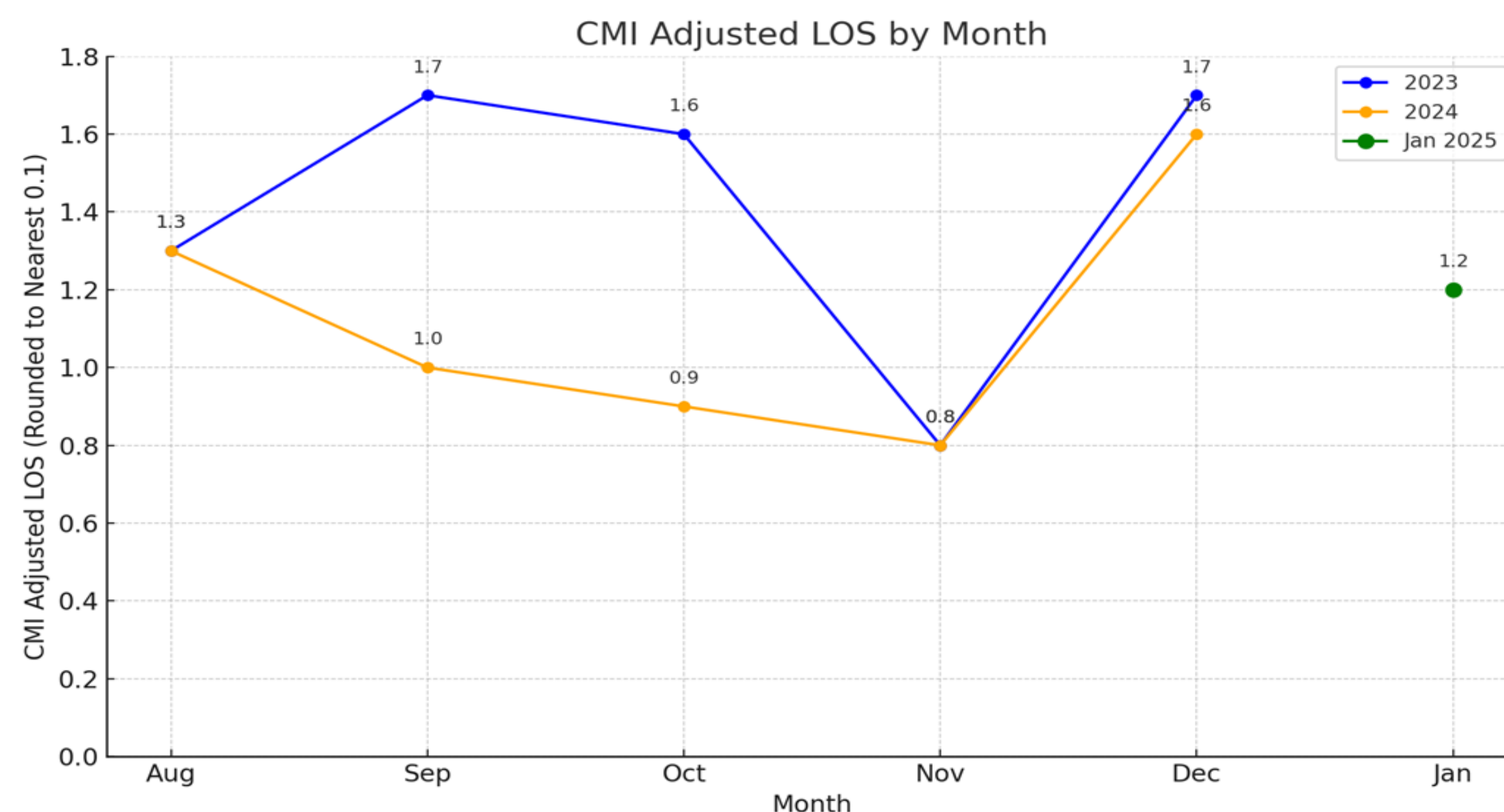


Figure 2. CMI Adjusted LOS by Month

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Discussion

Structured daily interdisciplinary rounds strengthened collaboration across nursing, medicine, pharmacy, dietary, therapy, infection control, and case management, thus improving quality outcomes. Therapy engagement and discharge planning contributed to decreased CMI-adjusted LOS.

The results align with published evidence showing interdisciplinary rounds improve quality measures and reduce LOS and hospital-acquired complications. Rounds were implemented at limited cost, utilizing existing staff and workflow reorganization.

This quality improvement initiative was conducted in a single ICU and results may not generalize to other settings. CLABSI and LOS reduction may also reflect concurrent quality initiatives.

Daily structured rounds are feasible, sustainable, and improve both quality and efficiency. This model offers a replicable framework across critical care and acute care settings while supporting hospital-wide initiatives, such as CLABSI and LOS reduction. Future implications include standardization of interdisciplinary rounds across the system.

Functional Outcome Measures

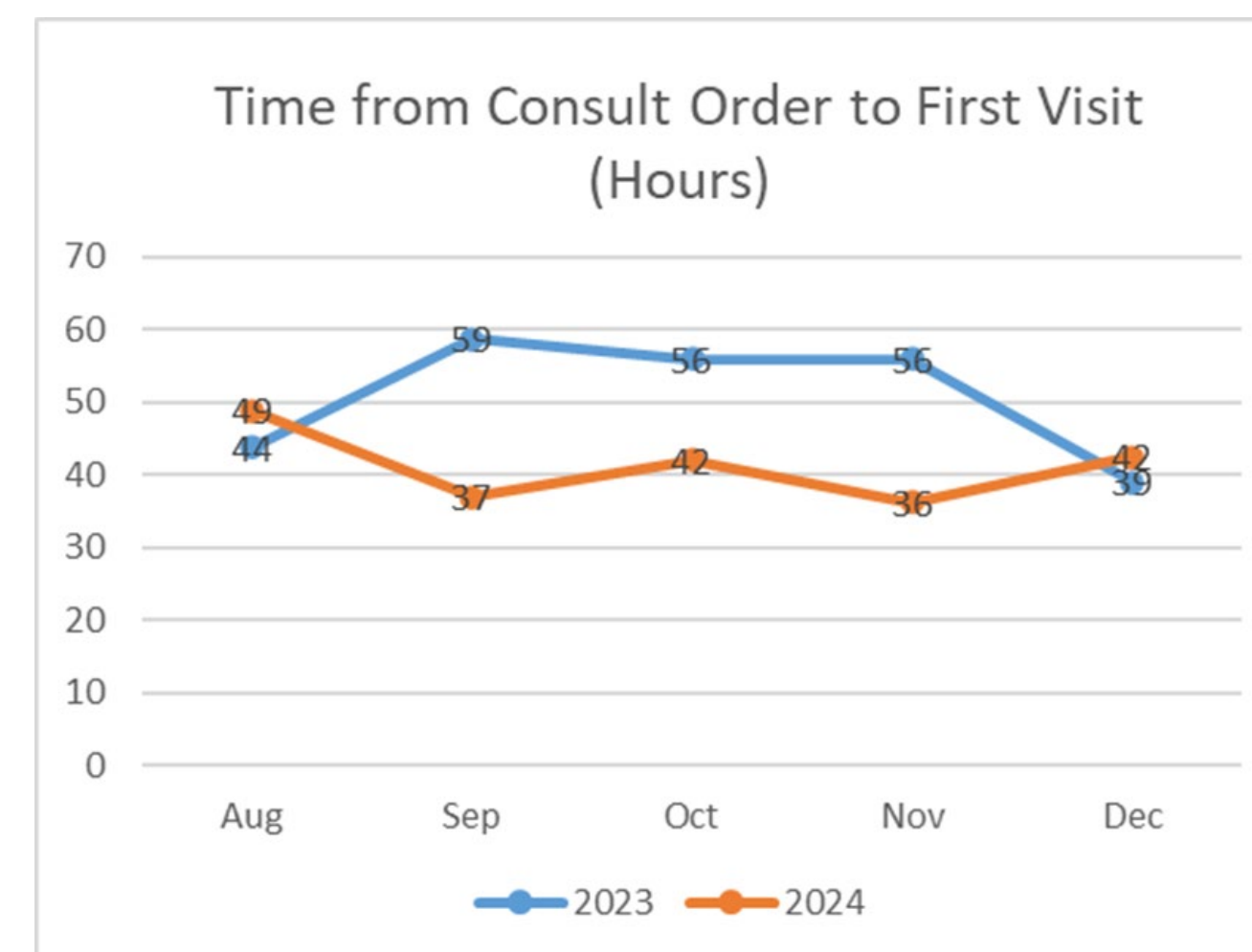


Figure 3. Time from Consult Order to First Visit

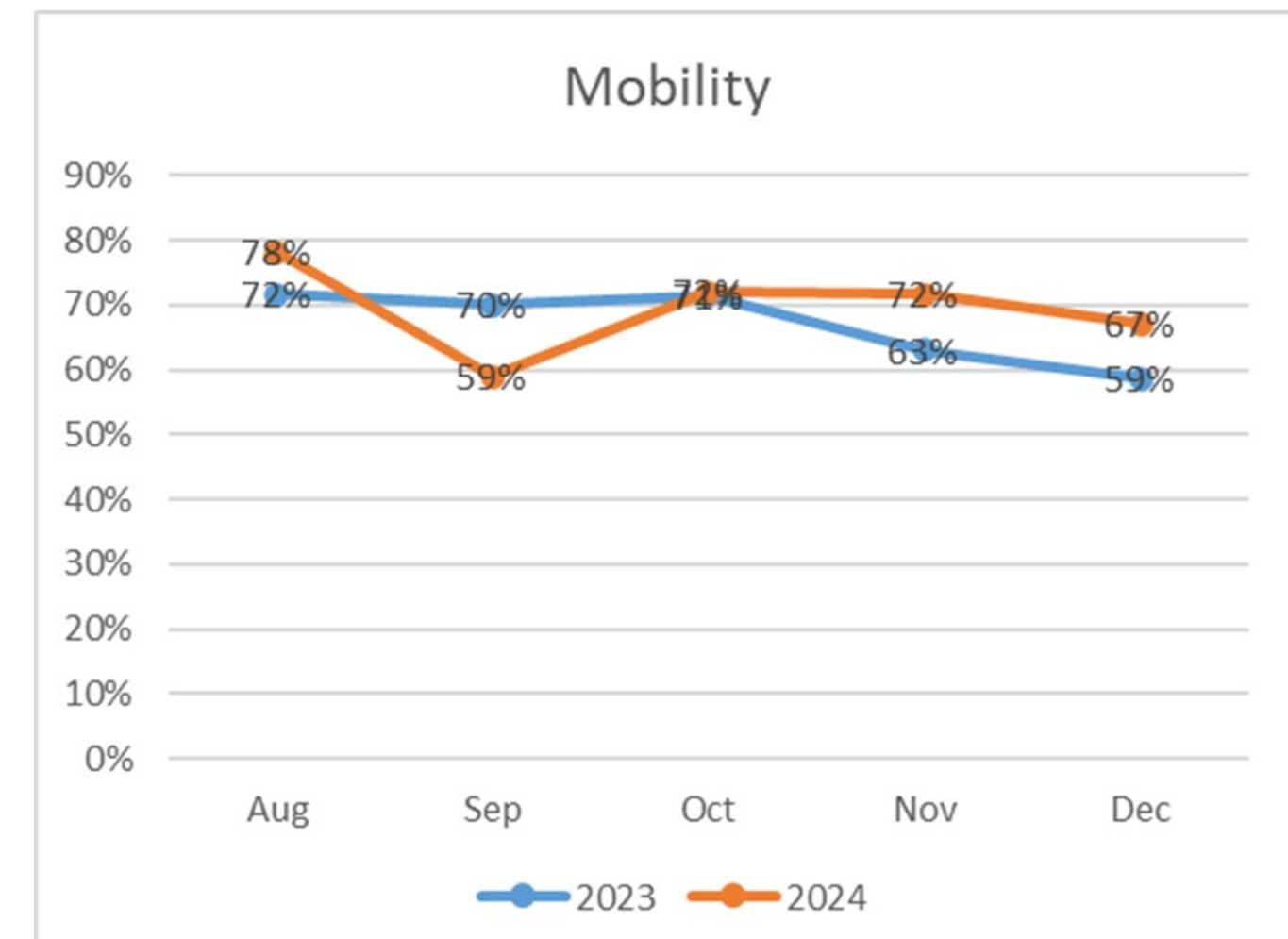


Figure 4. Mobility

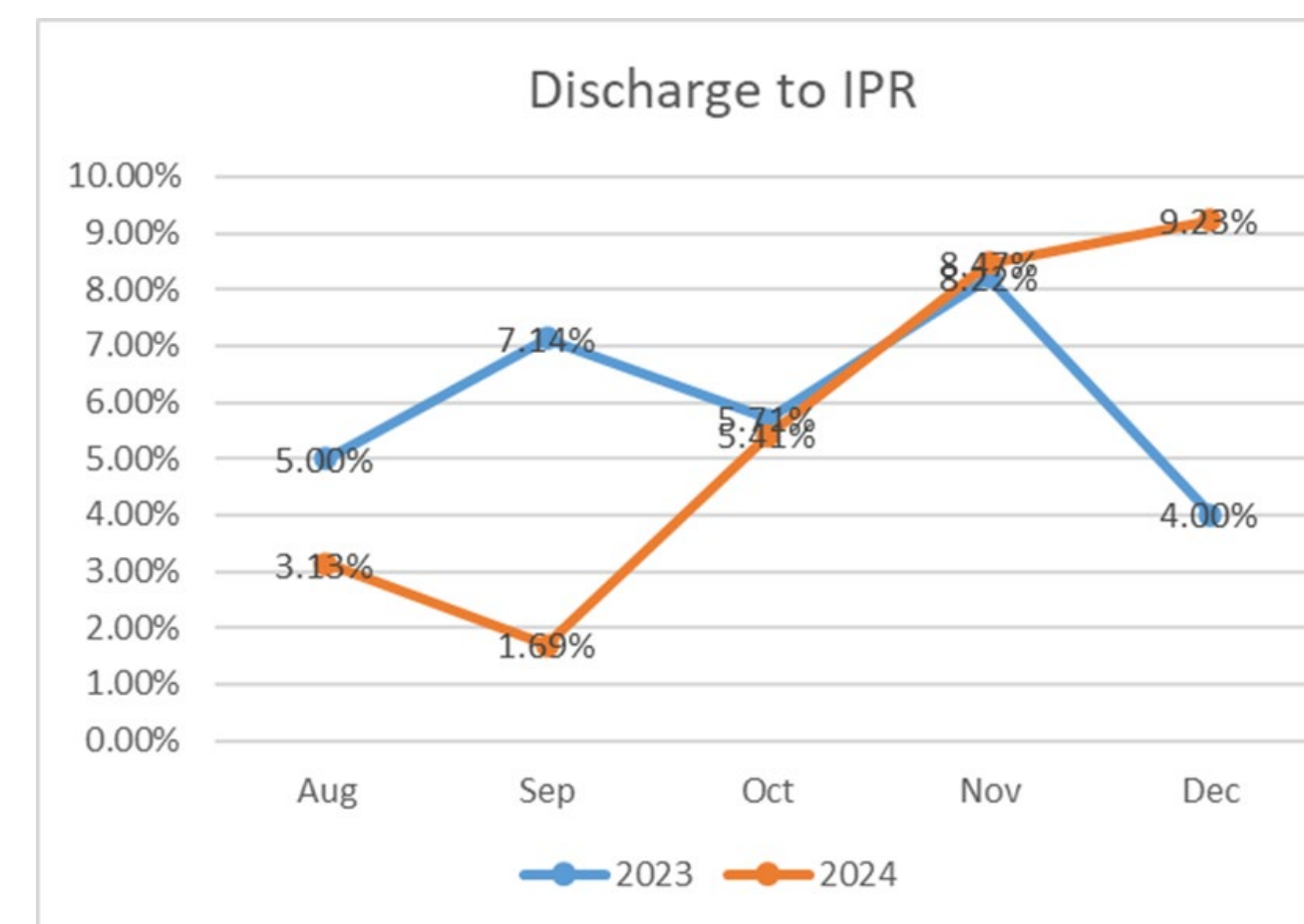


Figure 5. Discharge to IPR

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